

7.35.3 NMAC: proposed amendments outside the practicum. 1 of 4, Framework and defined terms

Working draft v1, July 26, 2026. Rule hearing August 28, 2026. Not a filing, not submitted, and not adopted rule text.

- 7.35.3.1 through 7.35.3.7. The defined-term dependency on 7.35.2.7 NMAC, and the four definitions the published rule itself supplies.
- This is one of four documents covering the twenty-four sections of 7.35.3 NMAC that the practicum amendment draft in *amendments/* does not reach. That draft covers 7.35.3.14, .18, .19, Paragraph (5) of Subsection H of .20, and a proposed .29, and nothing here reopens any of them.
- Every proposed change carries a citation to the provision, source document, or transcript it comes from. Where a defect has no fix that a source supplies, the published text is left exactly as it stands and the question is stated at that provision, with the choices and who decides.
- No figure appears in a right column unless a source carries it. A figure taken from elsewhere in the rule is badged with where it comes from.

The columns. Left is the rule as published, verbatim. Right is the proposed amendment: underlined green inserted, ~~struck red deleted~~, unmarked text carried forward unchanged. 60 from 7.35.3.15 B marks a figure taken from another provision of the same rule rather than newly drafted. A provision shown with no amendment is reproduced because it carries a finding.

7.35.3.2 Scope

1 OF 4 · FRAMEWORK AND DEFINED TERMS

CURRENT LANGUAGE	PROPOSED AMENDMENT
Proposed rule as published July 23, 2026	Draft for review. Not filed, not adopted
Entire section, scope 7.35.3.2 SCOPE: This rule applies to all patients, practitioners, clinicians, facilitators, healing centers and other approved locations, and educational programs who participate or seek to participate in the New Mexico medical psilocybin program.	Entire section, scope Not amended. This provision is reproduced because it carries a finding recorded below. SOURCE Not amended. Recorded for the terminology it fixes. 7.35.3.2 NMAC, page 1, lists the regulated classes as “patients, practitioners, clinicians, facilitators, healing centers and other approved locations, and educational programs”. Of those, 7.35.2.7 NMAC defines practitioner, clinician and approved location, and does not define facilitator, healing center or educational program. Carried over from the July 9, 2026 draft. Please review. The scope names six regulated classes, and 7.35.2.7 NMAC defines three of them. The rule uses “clinician” here and “certifying clinician” 53 times elsewhere, and 7.35.2.7 NMAC defines the first and not the second. Two choices. Leave the scope as published and rely on the definitions drafted at 7.35.3.7 NMAC to carry the undefined terms, or conform this section to the vocabulary the rest of the part uses. The Training and Education Committee and Department of Health staff decide which, and the choice governs the definition slate at 7.35.3.7 NMAC.

CURRENT LANGUAGE	PROPOSED AMENDMENT
Proposed rule as published July 23, 2026	Draft for review. Not filed, not adopted
Entire section, statutory authority 7.35.3.3 STATUTORY AUTHORITY: This rule is promulgated pursuant to the following statutory authorities: the New Mexico Department of Health Act, Subsection E of Section 9-7-6 NMSA 1978; and the Medical Psilocybin Act, Section 26-2D-7, NMSA 1978.	Entire section, statutory authority Not amended. This provision is reproduced because it carries a finding recorded below. ----- SOURCE Not amended. Recorded as a fact. 7.35.3.3 NMAC, page 1, cites "the Medical Psilocybin Act, Section 26-2D-7, NMSA 1978". Section 1 of the Act provides that Sections 1 through 11 of the act may be cited as the Medical Psilocybin Act. Section 7 of the Act is the section creating the program in the department and directing what the department shall establish. This record does not verify the codification. Carried over from the July 9, 2026 draft. Please review. This document does not verify any NMSA 1978 section number, and the codification asserted here has not been checked against the enacted text in this record. Two choices. Leave the citation as published, or ask the department to confirm the codification before the rule is filed. Department of Health staff and department counsel decide, and no board action is needed either way.

CURRENT LANGUAGE	PROPOSED AMENDMENT
Proposed rule as published July 23, 2026	Draft for review. Not filed, not adopted
Entire section, effective date 7.35.3.5 EFFECTIVE DATE: xx/xx, 2026, unless a later date is cited at the end of a section.	Entire section, effective date Not amended. This provision is reproduced because it carries a finding recorded below. SOURCE Not amended. Recorded for its interaction with the history notes. 7.35.3.5 NMAC, page 1, reads “xx/xx, 2026, unless a later date is cited at the end of a section”. Twenty-six of the twenty-eight history notes in the rule read “xx/xx/2026”. The notes for 7.35.3.27 and 7.35.3.28, page 19, read “9/22/2026”. Carried over from the July 9, 2026 draft, which read “XXXX,XX,2026”. Please review. Because 7.35.3.5 NMAC gives effect to a later date cited at the end of a section, the rule as published takes effect on an unfilled date except for 7.35.3.27 and 7.35.3.28, which take effect September 22, 2026. The rule hearing is set for August 28, 2026. No source in this record supplies the intended effective date, so none is drafted. Three choices. Conform the two history notes to the placeholder the other twenty-six carry, fill 7.35.3.5 NMAC with a date and leave the two notes as they stand if a later date for those two sections is intended, or fill all twenty-eight with one date. Department of Health staff decide, because the department sets the filing and effective dates.

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

Entire section, definitions

7.35.3.7 DEFINITIONS: The definitions in 7.35.2.7 NMAC apply to this part.

Entire section, definitions

7.35.3.7 DEFINITIONS: A. The definitions in 7.35.2.7 NMAC apply to this part.

B. In addition to the definitions in 7.35.2.7 NMAC, the following definitions apply to this part:

(1) “Certificant” means a practitioner, certifying clinician, facilitator, healing center or other approved location, or educational program that holds a certification issued by the department under this part.

(2) “Facilitator” means an individual who is certified by the department under this part to work alongside a practitioner during medical psilocybin services, under the direct supervision of a practitioner, and to provide peer support to qualified patients as well as logistical and administrative support to the practitioner and to the healing center.

(3) “Registrant of another approved location” means the practitioner or facilitator who holds a certification of an other approved location issued under Subsection B of 7.35.3.11 NMAC.

(4) “Student” means an individual who is registered with a psilocybin educational program certified by the department under this part.

SOURCE Four definitions are added. Each is drawn from a single provision of the rule as published, cited here, and none is composed for this draft. Certificant: 7.35.3.26 NMAC, page 16, which names the six classes that hold a certification. Facilitator: 7.35.3.13 NMAC Subsection B, page 8, which states the scope of work, together with 7.35.3.9 NMAC Subsection F, page 3, which requires certification. Registrant of another approved location: 7.35.3.11 NMAC Subsection B, page 6. Student: 7.35.3.20 NMAC Paragraph (5) of Subsection H, page 14. Section 7.35.3.7 as published reads in full: “The definitions in 7.35.2.7 NMAC apply to this part.” This section is new; the July 9, 2026 draft carried no definitions section. Addendum A maps every undefined term. Amending 7.35.2 NMAC is a separate rulemaking, because that part was adopted effective June 23, 2026, so any term this part needs and 7.35.2.7 NMAC does not supply has to be carried here.

Please review. Two questions here, and the second is the larger one. First, the slate. Sixteen terms that carry regulatory consequence in this part are defined nowhere: the four definitions drafted here are the only ones a single provision of the rule supplies, and twelve remain, including healing center, which appears 54 times, and certifying clinician, which appears 53 times. No source in this record defines either, so neither is drafted. Addendum A lists all sixteen with counts. The Training and Education Committee decides which to send to the department, and the department decides what each says. Second, the facilitator. Section 3(B) of the Medical Psilocybin Act defines a clinician as an approved health care provider licensed in New Mexico who holds a permit from the department to provide medical services to qualified patients. Section 5 of the Act exempts from arrest, prosecution and penalty a producer, a clinician and a qualified patient, and Paragraph (2) of Subsection B of Section 5 makes lawful the conduct of “a clinician administering or a qualified patient taking psilocybin in an approved setting”. A

CURRENT LANGUAGE

Proposed rule as published July 23, 2026

PROPOSED AMENDMENT

Draft for review. Not filed, not adopted

facilitator holds no professional license under this rule: Subsection F of 7.35.3.9 NMAC, page 3, requires no license of a facilitator applicant. Subsection B of 7.35.3.14 NMAC, page 9, nonetheless authorizes facilitators to possess medical psilocybin products and to provide them to qualified patients. Whether a facilitator falls within Section 3(B) of the Act, and so within the Section 5 exemption, is a question of statutory construction that this record cannot settle and that a definition in this part cannot cure if the answer is no. Two choices once it is answered: if a facilitator is within Section 3(B), nothing needs to change; if not, the authority in Subsection B of 7.35.3.14 NMAC has to be narrowed or the Act amended. Department of Health staff and department counsel decide it, and the board should be told the answer before the rule is filed. Two things bear on this and neither is in doubt. The practicum amendment draft reads the same Paragraph (2) of Subsection B of Section 5 against the same Subsection H of Section 3, which defines a qualified patient as “a patient whose clinician has judged the patient to be a medically appropriate candidate for the use of medical psilocybin based on being diagnosed with a qualifying condition”, and concludes that a participant who is not a qualified patient falls outside the provision; its proposed 7.35.3.29 accordingly states the statutory amendment it would need and conditions the whole section on that amendment. The question recorded here is the other half of the same provision, about who may administer rather than who may take. Separately, the position of a student is not in doubt and is not raised here: the practicum draft locates the provision of the medicine in the practitioner or the healing center under Subsections A and C of 7.35.3.14 NMAC, and 7.35.3.20 NMAC Subsection D, page 14, provides that “students completing their practicums may be present during an administration session”.

Addendum A Terms this part uses and no part defines

7.35.3.7 NMAC provides in full: "The definitions in 7.35.2.7 NMAC apply to this part." Every count in the table below is produced from the text layer of the published rule and of 7.35.2 NMAC each time this document is built, section by section, so no count in it is transcribed by hand. Column headings are section numbers within each part. The first block is terms used in 7.35.3 NMAC that 7.35.2.7 NMAC does not define. The second block, shaded, is the terms 7.35.2.7 NMAC does define, shown for contrast.

Addendum A

TERMS THIS PART USES AND NO PART DEFINES

Term in 7.35.3 NMAC	1	2	3	4	5	6	7	8	9	10	11	12	13	14	15	16	17	18	19	20	21	22	23	24	25	26	27	28	Part 3	Part 2
educational program	1					1				28		7			3	3	13	1	2	1	3					1	3		68	0
facilitator	1					1			5	5	3	1	14	5			2	15	4	3						1	4		65	0
healing center	1										10	1	2	10					1	21	3					1	3		54	0
certifying clinician						1		16	5	4			11					10								1	4		53	0
other approved location	1										6		1	3					1	14	3					1	3		34	0
practicum									3	4		1			1		5		13	2									29	0
student												5				3	12		5	3									28	0
certificant																					2	3	3	2			1		11	0
registrant																				10									10	0
didactic										4								4	2										10	0
adverse health event											1		1							4									6	0
administrative review committee																									4				4	0
graduate												1					2		1										4	0
simulated patient																	1	2	1										4	0
medical psilocybin services													1					1		1									3	0
equity and access fund								1																					1	0
Defined in 7.35.2.7 NMAC, for contrast																														
practitioner	1					1			5	5	3	1	12	6			2	14	6	3						1	5		66	12
clinician	1						5						4												1		2		13	4
qualified patient						1		3			2		4	3					3		3			1		1	1		22	6
administration session											5			3				1		7		1							17	4
approved location	1					1					6	1	1	3					1	15	3					1	3		37	2
guide																													0	3

What the table shows. Sixteen terms carrying regulatory consequence in this part are defined in neither part. Four of them are defined in the draft at 7.35.3.7 NMAC, because a single provision of the published rule supplies the content: certificant, facilitator, registrant of another approved location, and student. Twelve are not, and the two largest are healing center and certifying clinician. "Guide", the one defined term in 7.35.2.7 NMAC for an individual who assists practitioners during administration sessions, appears nowhere in 7.35.3 NMAC, while "facilitator", which 7.35.2.7 NMAC does not define, appears throughout it. 7.35.2 NMAC was adopted effective June 23, 2026, so amending 7.35.2.7 NMAC is a separate rulemaking and any term this part needs has to be carried in this part.

The Act. Section 3(B) of the Medical Psilocybin Act defines a clinician as an approved health care provider licensed in New Mexico who holds a permit from the department to provide medical services to qualified patients. 7.35.2.7 NMAC defines "Clinician" in the same terms except that it reads certification where the Act reads permit, and defines "Permit" as the authorization to operate as a psilocybin producer or psilocybin testing laboratory. Section 5 of the Act exempts a producer, a clinician and a qualified patient from arrest, prosecution and penalty. Neither the Act nor 7.35.2.7 NMAC uses the word facilitator. The consequence is stated at 7.35.3.7 NMAC in the redline above.

Addendum B Dependencies across the two drafting scopes

Provisions in these four documents that operate on, or are operated on by, a provision the practicum amendment draft in *amendments/* reaches. Nothing in these documents amends a provision in that draft's scope. Where a fix would require one, it is recorded here and left undrafted.

Addendum B

DEPENDENCIES ACROSS THE TWO DRAFTING SCOPES

Provision here	Provision in the practicum draft's scope	Relationship	Handled
7.35.3.11 A(11), page 5	7.35.3.14 C, page 9	7.35.3.14 C conditions healing center owner and employee authority on being registered with the department, and no registration exists. The registration is drafted here, at the point where the healing center already files the names.	Drafted here. The practicum draft flagged 7.35.3.14 C and did not amend it, and it is not amended here either.
7.35.3.7, page 1	7.35.3.20 H(5), page 14	The definition of student drafted here is taken from the qualified student test in 7.35.3.20 H(5). The practicum draft amends that paragraph for the permit title and does not change the test.	Drafted here, sourced there. If the test changes, the definition follows it.
7.35.3.7, page 1	7.35.3.14 B, page 9	7.35.3.14 B authorizes facilitators to possess psilocybin products and provide them to qualified patients. Whether a facilitator is a clinician under Section 3(B) of the Medical Psilocybin Act, and so within the Section 5 exemption, governs whether that authority holds. This is the administering half of Paragraph (2) of Subsection B of Section 5 of the Act. The practicum draft reads the taking half of the same provision against Subsection H of Section 3, and its proposed 7.35.3.29 states the statutory amendment that half would need and conditions the section on it, so the two drafts read the same provision consistently.	Not drafted. Recorded at 7.35.3.7 as a question for department counsel.
7.35.3.7, page 1	7.35.3.14 A and C, page 9, with 7.35.3.20 D, page 14	The position of a student is not in doubt and is not raised in these documents. The practicum draft locates the provision of the medicine in the practitioner or the healing center, and 7.35.3.20 D provides that "students completing their practicums may be present during an administration session". The definition of student drafted here does not disturb that.	Settled in the practicum draft. Nothing here contradicts it.
7.35.3.13 B, page 8	7.35.3.14 B, 7.35.3.19 C, 7.35.3.20 H(5)	The facilitator scope of work in 7.35.3.13 B is narrower than the authority the other three provisions confer, and "direct supervision" is not defined anywhere in the rule.	Not drafted. Any amendment has to be coordinated with the practicum draft.
7.35.3.17 A, pages 10 to 11	Proposed 7.35.3.19 H, which exists only in that draft's re-lettering of 7.35.3.19	Whether the consultation requirement is stated in hours or in cases, and whether it sits at the proposed 7.35.3.19 H or at 7.35.3.17 A, is open in the practicum document for Dr. Metz, Ms. Wilson and Dr. Leeman. As published, 7.35.3.19 NMAC runs A through G and has no Subsection H.	Not drafted. 7.35.3.17 A is left exactly as published so that their answer governs.
7.35.3.10 D(1), page 5	7.35.3.19 G(4), page 13, which that draft re-letters as 7.35.3.19 K	Two waivers of the practicum hours requirement each set a 40-hour floor for applications received by December 31, 2027. For the same 40 hours of contact time, 7.35.3.10 D(1) requires "two separate group sessions" and 7.35.3.19 G(4) requires "A minimum of one group session". The practicum draft records the conflict at its 7.35.3.19 K and amends neither provision, on the ground that 7.35.3.10 is outside its scope.	Not drafted here either. Conforming one to the other is a choice between two figures already in the rule, so neither document makes it and the conflict stands until the committee chooses.
7.35.3.16 A and C, page 10	7.35.3.18, pages 11 to 12	The third-party evaluation assesses the curriculum that 7.35.3.18 requires. Curing the evaluator conflict does not touch the curriculum, and the practicum draft's changes to the curriculum do not touch the conflict.	Drafted here. No coordination needed.
7.35.3.20 heading, page 13	7.35.3.20 heading, page 13	The heading reads 7.34.3.20. The practicum draft corrects it.	Not duplicated here. The correction stands in the practicum draft.

Addendum C Defects recorded and not drafted, and why

Every defect this record found in the twenty-four sections, that this draft does not amend. The reason in each row is the reason no language is proposed, not an assessment of severity. Each has a review note at the provision named, stating the choices and who decides.

Addendum C

DEFECTS RECORDED AND NOT DRAFTED, AND WHY

Provision	Defect	Why nothing is drafted
7.35.3.5, page 1, with the history notes at page 19	Twenty-six history notes carry the placeholder "xx/xx/2026"; the notes for 7.35.3.27 and 7.35.3.28 carry 9/22/2026. Under 7.35.3.5 a later date cited at the end of a section controls, so two sections would take effect before the rest has a date.	No source supplies the intended effective date, and which way to conform is the department's to choose.
7.35.3.7, page 1	Twelve of the sixteen undefined terms have no definition in any source, including healing center, 54 occurrences, and certifying clinician, 53 occurrences.	Drafting a defined term with regulatory consequence from no source would be composing it. Addendum A records the slate.
7.35.3.9 B, page 3	A renewal application is due no less than 30 days before expiry, and the department has no deadline to decide it, so a certification can lapse while a timely renewal is pending. Nothing continues the certification in the interim.	The fix is a continued-validity provision, which is a new grant, and no source in this record proposes its terms.
7.35.3.9 E(1) and F, page 3	A practitioner applicant must document a professional license "(e.g. PSY, LSW, LCSW)" with no stated scope of practice, and a facilitator applicant needs no license at all. The Wilson working redline comments "Also no scope here - what happened to therapy scope? It's missing - if DOH wants that, they need to include it." and "without specifying, they're setting themselves up to have athletic trainers and massage therapists apply and then not have a regulation to point to explain why that's not sufficient."	The redline raises the gap and supplies no list of qualifying licenses, and no other source in this record supplies one.
7.35.3.8 C, page 2, with 7.35.3.9 A(1), page 2	The department has 30 business days to decide a patient application and 30 calendar days to decide a certificant application.	Conforming one to the other is a choice between two figures already in the rule, and both bind the department.
7.35.3.10 A(2) and A(3), pages 4 to 5	The list of out-of-jurisdiction programs populates only when an individual application relying on a program is approved, so it is empty at launch. A(3) refers to programs approved by Oregon and Colorado "as of December 31, 2027", which is after the December 31, 2027 waiver in D(1) has closed.	Both fixes are new grants of authority to the department, and no source in this record proposes their terms.
7.35.3.10 D(1), page 5, with 7.35.3.19 G(4), page 13	The two 40-hour waivers set different group-session minimums, two against one.	Conforming one to the other is a choice between two figures already in the rule. The practicum draft records the same conflict and amends neither provision, so neither document makes the choice. Addendum B.
7.35.3.11 A(22)(d) and (e), and B(10) (d), pages 5 to 7	The 15-minute threshold for a remote natural environment. The Wilson working redline asks twice whether it should be 30.	A question in the source, not a recommendation, and no source supplies a figure.
7.35.3.11, page 5	The two-person universal requirement the Wilson working redline adds, which its author records did not reach the published rule.	The author records that she does not know where the provision belongs. Placing it would answer her question.
7.35.3.12 A(20) and B(1), pages 7 to 8	An application filed on or before December 31, 2027 may be approved without the third-party evaluation provided the evaluation is submitted by December 31, 2027, so the deferral shortens to nothing as that date approaches and is unavailable on the last day.	The fix is a period, and no source in this record supplies one. Tying it to the certification term would be a choice about how long a program may operate unevaluated.
7.35.3.12, pages 7 to 8, against the July 9, 2026 draft	The grounds for denying an educational program application present in the July 9 draft do not appear in the published rule. Finding M9.	Restoring deleted grounds is a substantive policy decision, and this record cannot show whether the deletion was deliberate.

Addendum C

DEFECTS RECORDED AND NOT DRAFTED, AND WHY

Provision	Defect	Why nothing is drafted
7.35.3.17 A, pages 10 to 11	The mentoring obligation, and whether it is stated in hours or cases and where it sits.	Open in the practicum document for three named people. Addendum B.
7.35.3.17 B, page 11	The test-out price cap is a fraction of the price of "each of the educational modules", which assumes per-module pricing. The Wilson working redline strikes the subsection.	Whether the option survives at all is the committee's decision, and a restated cap is worth drafting only if it does.
7.35.3.20 A, page 13, with 7.35.3.21 A, page 15	Healing centers must keep a roster of all qualified patients intended to use and who have used the location, and the department may interview patients.	Whether the roster should be kept in a form that does not identify patients turns on the department's data needs, which no source in this record states.
7.35.3.20, page 13	The term "registrant of another approved location", 10 occurrences, against the certification that 7.35.3.11 B issues.	The definition drafted at 7.35.3.7 ties the two words together. Conforming all 10 occurrences instead is a drafting choice, and this document does not make it.
7.35.3.21, page 15	No interval is set for department assessments, and none for repeating a third-party evaluation after the first.	An interval is a figure and a demand on department capacity. No source supplies either.
7.35.3.25 C and D(2), page 16	The administrative review committee decides a denied patient application and is constituted nowhere in 7.35.3 NMAC or 7.35.2 NMAC. Four occurrences, all in this section.	Only the department can say who the committee is.
7.35.3.25 E, page 16	"Except as otherwise provided by law, there shall be no right to judicial review of a decision by the administrative review committee." The Wilson working redline comments on it.	Whether the provision is within the department's authority is a legal question for department counsel.
7.35.3.24, page 15	The July 9, 2026 draft allowed a complaint from patients or staff; the published section allows one from a qualified patient or certificant, so staff who are neither have no route.	Restoring standing for staff is a policy decision. The confidentiality assurance the July 9 draft carried is restored in the redline above, because that text exists in a source.
7.35.3.27, pages 16 to 19	A certificant suspended immediately under Subsection A can be out of practice more than 135 days before a final decision, and there is no expedited track. Finding N5.	An expedited schedule is a set of periods binding the department and the secretary, and no source supplies them.
7.35.3.8 D, 7.35.3.25 A, 7.35.3.27 C(7), pages 2, 16 and 17	Three provisions describe review of a denial and their scopes overlap, so a patient applicant denied on the merits may have both an informal review and a hearing. Finding M23.	The redline fixes the name of the proceeding only. Which track applies to which denial is a question of what process is owed.

Out of scope by decision. The controlled-substance number requirement for certifying clinicians. It appears at Paragraph (3) of Subsection B of 7.35.3.8 NMAC, page 1, and at Paragraph (2) of Subsection D of 7.35.3.9 NMAC, page 3. Paragraph (2) is reproduced in the redline above because the renumbering of the paragraphs around it required it. It is reproduced as published and is not amended, and nothing is proposed about it.

Addendum D Mechanical defects across all twenty-eight sections

Numbering, dates and drafting form, checked across the whole of 7.35.3 NMAC rather than only the sections these documents amend. Every count and every entry is produced from the text layer of the published rule each time this document is built.

Addendum D

MECHANICAL DEFECTS ACROSS ALL TWENTY-EIGHT SECTIONS

Item	Count	Where	Corrected
Section headings reading 7.34.3 instead of 7.35.3	5 of 28	7.34.3.13, 7.34.3.14, 7.34.3.20, 7.34.3.23, 7.34.3.25	7.35.3.13 in the document 4 above; 7.35.3.14 in the practicum draft; 7.35.3.20 in the practicum draft; 7.35.3.23 in the document 4 above; 7.35.3.25 in the document 4 above
History notes carrying a real effective date rather than a placeholder	2 of 28	7.35.3.27 reads 9/22/2026, 7.35.3.28 reads 9/22/2026	Not corrected. Recorded at 7.35.3.5 above.
History notes omitting the "- N" new-section designator	9 of 28	7.35.3.11, 7.35.3.13, 7.35.3.14, 7.35.3.20, 7.35.3.22, 7.35.3.23, 7.35.3.24, 7.35.3.25, 7.35.3.26	Not corrected. Mechanical, and the designator is the department's to set.
Numbering gap inside a subsection	1	7.35.3.9 D runs (1), (2), (4), (5), (6). There is no (3).	Renumbered in document 4 above.
Subsections lettered "(A)" rather than "A."	4	7.35.3.11 A, and 7.35.3.14 A, B and C. Letters found: A, B, C	7.35.3.11 A in document 3 above. 7.35.3.14 in the practicum draft.
Sentence with no verb governing the decision	1	7.35.3.25 D(1), page 16. Carried over from the July 9, 2026 draft.	Corrected in document 4 above.
Sentence that does not complete	1	7.35.3.18 F, page 12.	In the practicum draft's scope. Corrected there.
Presiding official named inconsistently	1	7.35.3.27 M reads "hearing examiner"; the rest of 7.35.3.27 reads "hearing officer".	Corrected in document 4 above.
Duplicated words	1	7.35.3.24 reads "the electronic system designated by the department electronic system".	Corrected in document 4 above.
Spacing inside a heading	1	7.35.3.16 C reads "Conflict of -interest prohibitions". The July 9, 2026 draft read "Conflict-of-interest prohibitions".	Corrected in document 2 above.
Class named in the lead-in but not in the operative words	1	7.35.3.27 B(8) opens "for certifying clinicians and practitioners" and then refers only to "the clinician".	Corrected in document 4 above.
Patient described as certified rather than enrolled	1	7.35.3.27 C(1) reads "a certified patient".	Corrected in document 4 above.

Sources. Rule as published: *docs/documents/rules-draft-2026-07-23-published.pdf*, 19 pages, 7.35.3.1 through 7.35.3.28. Prior board-meeting draft, used for new against carried-over determinations: *docs/documents/rules-draft-2026-07-09.pdf*. Defined terms: 7.35.2 NMAC as adopted effective June 23, 2026, at *source-text/7.35.2-NMAC-adopted-2026-06-23.txt*. Statute: the Medical Psilocybin Act, Senate Bill 219, 57th Legislature, First Session, 2025, as passed; Section 1 provides that Sections 1 through 11 of the act may be cited as the Medical Psilocybin Act, and those section numbers are the ones cited here. This record does not verify any NMSA 1978 section number. Working redline: Denali Wilson, NMAC 7.35.3, July 25, 2026, tracked changes and comments as exported from the document file. July 17, 2026 transcripts, morning Advisory Board and afternoon Training and Education Committee, both labeled "UNOFFICIAL AUTO-GENERATED TRANSCRIPT. NO SPEAKER ATTRIBUTION."; no speaker is named from either transcript in this document. Inventory relied on and re-verified: *analysis/july23-rule-concerns.md*.

Method. The left column reproduces the rule as published, verbatim, with line breaks introduced by the PDF collapsed to single spaces and hyphenation introduced by a line break rejoined. Nothing else is

altered. Every block was checked against the text layer of the published rule by exact contiguous match, and the build aborts if any block fails. *amendments-remainder/audit.py* re-checks every verifiable claim in these documents and exits non-zero on any failure; *amendments-remainder/AUDIT.md* is its output.

Status. Working draft v1, document 1 of 4. Not a filing, not submitted, and not adopted rule text. Nothing in *amendments/* or *docs/* was modified by this work.